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Safe Childbirth Practice Guidelines

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1. Introduction

Recent WHO guidelines emphasize safe childbirth practices to enhance maternal and neonatal outcomes worldwide. This abstract explores key recommendations for healthcare providers, focusing on evidence-based interventions. Essential components include skilled birth attendance, promoting timely referrals, and ensuring hygienic conditions to prevent infections. Additionally, the guidelines stress the importance of continuous monitoring of maternal and fetal well-being during labor, utilizing partograph charts to track progress and identify deviations promptly.

The abstract underscores the significance of respectful maternity care, advocating for women's autonomy and informed decision-making throughout the childbirth process. It highlights the implementation of non-pharmacological pain management strategies and the judicious use of interventions like oxytocin for labor augmentation. Furthermore, the guidelines emphasize the role of postnatal care in preventing complications and promoting breastfeeding.

In conclusion, adherence to WHO's safe childbirth practices is crucial for reducing maternal and neonatal mortality rates globally. By integrating these recommendations into healthcare systems, countries can strive towards achieving sustainable development goals related to maternal and child health, ensuring safer childbirth experiences for women and their infants.

How to practice safe childbirth:

Childbirth is a complex process and it is very important to remember to not miss out on

crucial steps. The WHO has formulated certain checklists to help health-care workers ensure that essential birth practices are performed at critical moments during childbirth for every delivery, every time. While it is not possible to list on a single checklist all practices that are required at each birth, the Checklist does list a core set of practices that have been proven to reduce harm to mothers and new-borns.

The four sections, or pause points, are specific points in time when birth attendants should ■check■ that they have completed essential birth practices. These pause points allow birth attendants to make their ■checks■ at times when they can protect the mother and new-born against dangerous complications, but the pause points also take place when it is convenient for birth attendants to take the time to perform the checks.

The 4 Pause Points:

1. On admission-

2. Just before pushing (or before Caesarean)

3. Soon after birth (within one hour)

4. Before discharge

On admission:

- To detect and treat complications that she may already have
- To prepare her (and her companion) for labour and delivery
- To educate her (and her companion) about danger signs for which she should call for help
- Takes obstetric, medical and surgical history
- Assesses gestational age correctly -Assesses gestational age through either LMP or Fundal height or USG (previous or present is available)
- Records fetal heart rate – Fetoscope / stethoscope / doppler
- Records mother's BP and temperature

Just before or during pushing/ caesarean:

- To detect and treat complications that can occur during labour
- To prepare for routine events and possible crisis situations that may occur after birth.
- All caesarean should be classified as per modified Robsons criteria
- Provider ensures six ■cleans' while conducting delivery -Sterile gloves , cord clamp , betadine solution ,sterile cutting edge (blade/scissors)
- Performs episiotomy only when indicated with the use of appropriate local anaesthetic.
- Provider allows spontaneous delivery of head by flexing it and giving perineal

support, manage cord round the neck, assists delivery of shoulders and body.

- Delivers the baby on mother's abdomen
- AMTSL – Active management of third stage of labour.
- Ensures immediate drying, and assess breathing
- Performs delayed cord clamping and cutting unless indicated otherwise.
- Ensures early initiation of breastfeeding within 1 hour of birth.
- Assesses the new-born for any congenital anomalies
- Weighs the baby and administer Vitamin K

Soon after birth (within one hour):

- To detect and treat complications that can occur after delivery
- To educate the mother (and her companion) about danger signs for which she should call for help.

- Assesses uterine tone and bleeding per vaginum regularly
- Identifies shock by signs and symptoms (pulse > 110 per minute, systolic BP < 90 mmHg, cold clammy skin, respiratory rate > 30 per minute, altered sensorium and scanty urine output < 30 ml per hour)

- Ensures availability of wide bore cannulas (No. 14/16), IV infusion sets and fluids and containers for collection of blood for haemoglobin, blood grouping and cross matching

- Shouts for help, follows ABC approach, monitors vitals, elevates the foot end and keeps the woman warm

- Starts IV infusions, collects blood for Hb and grouping and cross matching, catheterizes the bladder and monitors I/O, gives oxygen at the rate of 6-8 litres per minute .

- Identifies specific cause of PPH .
- Newborn resuscitation

Before discharge:

- To be sure that the mother and newborn are healthy before discharge
- Follow-up has been arranged
- Family planning options have been discussed and offered to the mother (and her companion)

- Education on danger signs to look out for, both in the mother and her baby, has been given in case immediate skilled care is needed.

The WHO checklists are attached to the case files of all the mothers so that it is followed at all the pause points irrespective of the location.

Refer annexure I for WHO checklists.

SAFE BIRTH SUPPLIES-

Supplies required to safely and hygienically manage the process of labour and delivery.

- Ensures sterile/ HLD delivery tray is available
- Ensures functional items for newborn care and resuscitation
- Ensure availability of uterotonic agents - IM/IV oxytocin (preferred), misoprostol
- Designated new born corner is present
- Ensures functional items for newborn care and resuscitation
- Switches radiant warmer ■on' 30 min. before childbirth

GENERAL SUPPLIES AND EQUIPMENT

Power Supply

Clean Water

Soap or Alcohol Hand Rub

Disinfectant

Autoclave

Clean Gloves

Stethoscope

Thermometer

Blood Pressure Instrument

Partograph

Fetoscope/Doppler

SUPPLIES IN THE DELIVERY ROOM

Suction Machine

Mucus Extractor

Neonatal Bag and Mask

Oxygen Cylinder/Concentrator

Baby Scale

Needle/Syringe

Urine Dip Sticks

Sterilized Blade/Scissor

Cord Tie/Clamp

Clean Pads for Mother

Clean Towel

MEDICATIONS/INJECTIONS/DRIPS

Bag of IV Fluids

Injectable Oxytocin

Injectable Magnesium Sulphate

Antibiotics for Mother

Antibiotics for Infant

Antihypertensives

Labour room is maintained as per SOP of LaQshya.

(Refer to SOP of Department of Obstetrics and Gynaecology)

CHECK-1 On Admission

Does Mother need referral? Refer to HRU/Higher centre if any of following danger signs are present, mention reasons and given

- o Yes, organized

treatment on transfer note:

- o No
- o Vaginal bleeding
- o Severe abdominal pain
- o High fever
- o History of heart disease or other major illness
- o Severe headache or blurred vision
- o Difficulty in breathing
- o Convulsions

Partograph started? Start when cervix "4 cm, then cervix should dilate *1 cm/hr

- o Yes

- o No: will start when 4cm - Every 30 min: Plot maternal pulse, contractions,

FHR and colour of amniotic fluid

- Every 4 hours: Plot temperature, blood pressure, and cervical dilation in cm

NO OXYTOCIN/ other uterotonics for unnecessary induction/ augmentation of labor

Does Mother need Give antibiotics to Mother if:

- Antibiotics? o Mother's temperature 38°C (*100.5oF)

- o Foul-smelling vaginal discharge.

o Yes, given o Rupture of membranes > 12 hrs without labour or >18 hrs with labour

- o No

- o Labour >24 hrs or obstructed labour

o Rupture of membranes <37 weeks of gestation

Inj Magnesium Sulphate? Give first dose of inj. magnesium sulphate and refer immediately to FRU/Higher centre OR give full dose

- o YES, given

(loading and then maintenance) if at FRU if:

Mother has systolic BP *160 or diastolic *110

with +3 proteinuria OR BP systolic 140 or
diastolic *90 with proteinuria trace to +2 along
with any of:

Presence of any symptom like:

- o Severe headache
- o Pain in upper abdomen
- o Oliguria[<400ml urine in 24hr]
- o Blurring of vision.
- o Difficulty in breathing.
- o Convulsions

Corticosteroid Give corticosteroids in antenatal period (between 24 t
to 34. weeks) to mothers if:

- o Yes, given
- o No o True pre-term labour
- o Conditions that lead to imminent delivery like

APH, Preterm Premature ROM, Severe PE/E

Dose: Inj. Dexamethasone 6 mg IM 12 hourly

- total 4 doses

HIV status of the mother: If HIV+ and in labour:

- o Positive o If mother is on ART, continue same
- o Negative o If not on ART, start ART
- o If ART is not available, refer immediately

after delivery to ICTC/ART Centre/Link ART

Centre for further HIV management

If HIV status unknown:

- o Follow universal

precautions

- o Recommend HIV testing

Encouraged birth companion to be present during labour, at birth and till discharge:

Yes No

Are soap, soap, water, gloves available?

- o Yes, I will wash hands and wear gloves for each vaginal exam
- o No, supplies arranged.
- o Confirm if mother or Explain to call for help if there Counsel Mother

companion

is: and birth

will call for help

- o Bleeding companion on:

during

o Severe abdominal pain o Support to labour if needed

o Difficulty in breathing cope up
o Severe headache or with labour blurring vision pains.

o Urge to push o No bath/oil
o Can't empty bladder for baby every 2 hours o No pre lacteal feed

o Initiate breastfeedi
ng in half
an hour.

o Cloth and
wrap the
baby.

Name of the Provider: Date:

Signature:

CHECK-2 Just Before and During Birth (or C Section)

Does Mother need: Give Antibiotics to mother if any of the following are present:

o Antibiotics? o Mother temperature 38oC or 105.5oF
o Yes, given o Foul smelling vaginal discharge
o No o Rupture of membrane >18 hours with labour
o Labour >24 hours or obstructed labour now
o Caesarean section
• Inj. Magnesium Give first dose of inj. magnesium sulphate and refer sulphate? immediately to FRU/Higher centre OR give full dose (loading and then maintenance) if at FRU if: Mother has systolic BP

o Yes, given
>160 or diastolic >110 with +3 proteinuria OR BP systolic

o No
>140 or diastolic >90 with proteinuria trace to +2 along with
any of :

o Presence of any symptom like:
Severe headache Blurring of vision
Difficulty in breathing Pain in upper
abdomen Oliguria (passing <400 ml urine in 24 hrs).

o Convulsions
o Skilled assistant identified and ready to help at birth if needed

Confirm essential supplies Prepare to care for mother immediately after birth of baby
are at bedside/labour room: (AMTSL)*

For Mother o Confirm single baby only (rule out multiple
babies)

- o Gloves o Give inj. oxytocin 10 units IM within 1 minute
- o Soap and clean water o Do controlled cord traction to deliver placenta
- o Oxytocin 10 units in o Massage uterus after placenta is delivered, check
syringe for completeness (all Cotyledons and Membranes)

- o Pads for mother

For Baby Prepare to care for baby immediately after birth .

- o Two clean dry, warm o Dry baby, wrap, and keep warm, give Vit. K, start
towels breastfeeding

- o Sterile scissors/blade

- o If not breathing: clear airway and stimulate
to cut cord

- o If still not breathing:

- o Mucus extractor -Cut cord

- o Cord ligature -Ventilate with bag-and-mask

- o Bag-and-mask - Call for help (Paediatrician/SNCU/NBSU/F-
IMNCI trained doctor if available)

Name of the Provider: Date:

Signature:

CHECK -3 Soon After Birth (Within 1 hour)

Is Mother If bleeding \$500 ml, or 1 pad soaked in <5 min:

bleeding

- o Call for help, massage uterus, start oxygen, start IV fluids,
abnormally?

start oxytocin drip 20 units in 500 ml of RL@40-60

drops/min, treat cause

- o Yes,

- o If placenta not delivered or Or completely retained: give IM
shout for

or IV Oxytocin, stabilize and refer to FRU/Higher centre

help,

- o If placenta is incomplete: remove if any visible pieces, and
refer if refer immediately to FRU/ higher centre

needed or

treat if

facilities

available

☐ No

Does Mother Give antibiotics to mother if manual removal of placenta is need: performed, or if mother's temperature $\geq 38^{\circ}\text{C}$ ($\geq 100.5^{\circ}\text{F}$) and any of:

Antibiotics? ☐ Chills

☐ Foul-smelling vaginal discharge

☐ Yes,

given ☐ Rupture of membranes >18 hrs during labour

☐ No ☐ Labour was >24 hours

Inj. Magnesium Give first dose of inj. magnesium sulphate and refer immediately to sulphate? FRU/Higher centre OR give full dose (loading and then maintenance)

if at FRU if Mother has systolic BP >160 or diastolic >110 with $+3$

☐ Yes,

given proteinuria OR BP systolic >140 or diastolic >90 with proteinuria

☐ No trace to $+2$ along with any of:

☐ Presence of any symptom like:

Severe headache Blurring of vision Difficulty in

breathing

Pain in upper abdomen Oliguria (passing <400 ml urine in

24 hrs)

☐ Convulsions

Does Baby need: Give baby antibiotics if antibiotics were given to mother, or if baby has any of:

Antibiotics?

☐ Breathing too fast ($>60/\text{min}$) or too slow ($<30/\text{min}$)

☐ Yes,

given ☐ Chest in-drawing, grunting

☐ No ☐ Convulsions

☐ Looks sick (lethargic or irritable)

☐ Too cold (baby's temp $<36^{\circ}\text{C}$ and not rising after warming)

☐ Too hot (baby's temp $>38^{\circ}\text{C}$)

☐ Excessive crying

Referral?

☐ Yes, organized

☐ No

Refer baby to NBSU/SNCU/FRU higher centre if:

☐ Any of the above (antibiotics indications)

☐ Baby looks yellow, pale or bluish

Special care and Arrange special care/monitoring for baby if any of the following is monitoring? present:

- o Yes, o Preterm baby

organized

- o Birth weight <2500 g
- o No
- o Needs antibiotics
- o Required resuscitation

Syrup Give if mother is HIV+:

Nevirapine

- o If mother has received >24 weeks of ART, give syrup

Nevirapine to baby for 6 weeks

- o Yes,
- o If mother has received <24 weeks of ART or mother is not

given and

on ART, give syrup Nevirapine to baby for 12 weeks

will

continue

up to 6

weeks

- o NO
- o Started breastfeeding. Explain that colostrum feeding is important for baby.
- o Started skin-to-skin contact (if mother and baby well) and KMC in pre-term and

low-birth weight babies

- o Explain the danger signs and confirm mother/companion will call for help if danger signs present.

Name of the Provider: Date:

Signature:

CHECK-4 Before Discharge

Is Mother's bleeding controlled?

- o Yes
- o No, treat, observe and refer to

FRU higher centre if needed

Does mother need antibiotics? Give antibiotics to mother if mother has

temperature 38oC or 100.5oF with any of

- o Yes, give and delay discharge

:

- o No
- o Chills
- o Foul smelling vaginal discharge
- o Lower abdominal tenderness

Does baby need antibiotics? Give baby antibiotics if baby has any of:

- o Yes, give, delay discharge and refer o Breathing too fast ($>60\text{Vmin}$) or to FRU/ higher centre too slow ($<30\text{imin}$)

- o No o Chest in-drawing, grunting

- o Convulsions

- o Looks sick (lethargic or irritable)

- o Too cold (baby's temp $<36^{\circ}\text{C}$ and not rising after warming)

- o Too hot (baby's temp $>38^{\circ}\text{C}$)

- o Stopped breastfeeding

- o Umbilical redness extending to skin or draining pus

Is baby feeding well?

- o Yes, encourage mother for exclusive breastfeeding for 6 months

- o No, help mother, delay discharge; refer to NBSU/ SNCU/ Higher centre if needed

- o Discuss and offer family planning options to mother

- o Confirm post-delivery stay at facility for 48 hours in normal delivery and 7 days in C-section cases

- o Explain the danger signs and confirm mother/companion will seek help/ come back if danger signs are present after discharge

- o Arrange transport to home and follow-up for mother and baby

Thank mother for availing services from you

Danger Signs

Mother has any of: Baby has any of:

- o Excessive bleeding o Fast/difficulty breathing

- o Severe abdominal pain o Fever

- o Severe headache or visual

- o Unusually cold disturbance

- o Stops feeding well

- o Breathing difficulty

- o Less activity than normal

- o Fever or chills

- o Whole body becomes yellow

- o Difficulty emptying bladder

- o Foul smelling vaginal discharge

CONCLUSION

This guideline aims for respectful maternal care and safe childbirth and recovery in the post-

natal period.

ACKNOWLEDGEMENT

1. Who Safe Childbirth Checklist

2. National Health Mission Safe Childbirth Checklist.

Name of the Provider: Date:

Signature: